AMERICAN CLINICAL LABORATORY ASSOCIATION

February 20, 2026

Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy
Assistant Secretary for Technology Policy (ASTP) and the Office of the National Coordinator for Health Information Technology (ONC)
Attention: HHS Health Sector AI RFI
Mary E. Switzer Building
Washington, DC 20201

Submitted electronically

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care

Dear Mr. Posnack,

The American Clinical Laboratory Association (ACLA) appreciates the opportunity to respond to ASTP/ONC's request for information on accelerating the adoption and use of AI as part of clinical care, pursuant to the HHS Artificial Intelligence (AI) Strategy[1] in response to Office of Management and Budget (OMB) Memorandum 25-21[2].

ACLA is the national trade association representing leading laboratories that deliver essential diagnostic health information to patients and providers by advocating for policies that expand access to the highest quality clinical laboratory services, improve patient outcomes, and advance the next generation of personalized care. ACLA appreciates the Administration's efforts to identify regulatory, reimbursement, and research and development barriers in order to support rapid adoption and use of AI in clinical care. These efforts are critical to ACLA members' patient-centered mission, to improve the health of all Americans. As clinical laboratories strive to empower patients with actionable information about their health, AI is enhancing efforts to prevent and detect the onset of diseases and changes in chronic conditions, providing useful data to inform care in new ways.

The diagnosis and decision-making process is still reliant on the expertise of trained healthcare professionals, but AI-generated insights that take into account patient history, diagnostic test results, and medical records can assist clinicians with making a comprehensive diagnosis and formulating a treatment plan for each patient. The value of AI developments in laboratory medicine is based on AI's ability to quickly analyze large amounts of data, identify trends and patterns, and produce accurate insights that can support clinical decision making. In clinical laboratories, AI has the potential to help

[1] https://www.hhs.gov/press-room/hhs-unveils-ai-strategy-to-transform-agency-operations.html.
[2] https://www.whitehouse.gov/wp-content/uploads/2025/02/M-25-21-Accelerating-Federal-Use-of-AI-through-Innovation-Governance-and-Public-Trust.pdf.

laboratorians prioritize and streamline their work to increase efficiency and develop individualized treatment plans, and it can also help to increase testing volumes, enabling laboratories to serve more patients and to serve them better. AI already is being used to help improve business operations, streamline workflow, and ease administrative burdens.

As the Administration works to implement the President's AI Action Plan[3], recent Executive Orders[4] on AI, and OMB AI memoranda[5], ACLA respectfully requests the Administration consider the issues set forth below to maximize the benefits of AI technology.

Regulation: Federal Preemption and Regulatory Clarity

Preemption: The rapid development and proliferation of AI in healthcare underscore the need for a careful balance of regulatory clarity and stability—so that industry has a clear understanding of government expectations—while also maintaining the flexibility needed to adapt to evolving technological advancements and emerging innovations. Key to this careful balancing act is preempting a patchwork of state and local laws regulating healthcare AI, which would threaten to stifle innovation. ACLA appreciates the Administration taking the lead on this issue to ensure clarity and foster innovation.

Privacy: Under the current Health Insurance Portability and Accountability Act (HIPAA) requirements and its implementing regulations at 45 CFR Part 164 (the "Privacy Rule"), clinical laboratories are advancing AI innovation through the permissions granted to de-identify Protected Health Information (PHI) and use such de-identified information in a manner unrestricted by HIPAA, including to develop and train AI models. In its guidance on de-identification, the Office for Civil Rights expresses how de-identification supports the public good: "The process of de-identification, by which identifiers are removed from the health information, mitigates privacy risks to individuals and thereby supports the secondary use of data for comparative effectiveness studies, policy assessment, life sciences research, and other endeavors."[6] Clinical laboratories have contributed exponential advancements in AI-enabled medicine in recent years – from more precise identification of the origin of a patient's cancer, to better understanding and predicting a patient's response to specific treatments– thanks to research involving large datasets of de-identified information. These advancements simply would not have been possible without de-identification permissions under HIPAA.

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[3] https://www.whitehouse.gov/wp-content/uploads/2025/07/Americas-AI-Action-Plan.pdf.
[4] https://www.ai.gov/#resources-anchor.
[5] See: https://www.whitehouse.gov/wp-content/uploads/2025/02/M-25-21-Accelerating-Federal-Use-of-AI-through-Innovation-Governance-and-Public-Trust.pdf and https://www.whitehouse.gov/wp-content/uploads/2025/02/M-25-22-Driving-Efficient-Acquisition-of-Artificial-Intelligence-in-Government.pdf and https://www.whitehouse.gov/wp-content/uploads/2025/12/M-26-04-Increasing-Public-Trust-in-Artificial-Intelligence-Through-Unbiased-AI-Principles-1.pdf.
[6] See https://www.hhs.gov/hipaa/for-professionals/special-topics/de-identification/index.html.

ACLA hopes the Administration will continue to foster and encourage use of de-identified information. The Administration should take care not to disrupt the balance between genetic information and de-identification that already exists under the Privacy Rule. OCR guidance dating back to 2002 has confirmed that genetic information is PHI only if it is 'individually identifiable', indicating that OCR does not treat *all* genetic information as PHI.[7] This means that genetic information is not PHI if it is otherwise de-identified per the HIPAA Privacy Rule. The inclusion of genetic information in de-identified data sets has been instrumental in the medical advancements described above.

<u>CLIA Regulation Updates</u>: Reinforcing that the Clinical Laboratory Improvement Amendments (CLIA) is the regulatory framework for clinical laboratory services — including laboratory services that examine and analyze digital laboratory information—is essential for advancing innovation and the adoption of AI in clinical care. In 2025, ACLA secured a landmark victory in *ACLA v. FDA*, Civil No. 4:24-CV-479-SDJ (Mar. 31, 2025), which vacated a Biden-era rule to regulate laboratory developed testing services (LDTs) as medical devices, and affirmed that LDTs are professional services that should not be subject to an unsuitable Food and Drug Administration (FDA) regulatory framework. This ruling safeguards patient access to vital testing services and prevents the imposition of unnecessary, burdensome regulations that could undermine the effectiveness of the clinical laboratory system in the United States.

There remains an opportunity to make targeted updates to the CLIA regulations that would encourage development and adoption of AI technologies that enhance clinical diagnostics. These regulations were promulgated decades ago, before the proliferation of software and AI that have enhanced diagnostic practices and workflows. The following updates to CLIA would clarify that laboratory operations include analysis and examination of digital laboratory information (also referred to as "algorithmic-only" or "dry lab" work) and free pathologists and laboratory personnel to review such information remotely under the primary site's CLIA certificate.

<u>**First**</u>, the definition of "laboratory" under the CLIA regulations should be amended to expressly state that laboratories include facilities that perform "algorithmic" analysis of "patient-specific digital laboratory information," such as previously sequenced DNA or digital images of slides. Advancements in AI and clinical laboratory science have enabled laboratories to develop custom, algorithmic-only services that analyze digital laboratory information to unlock new clinical insights that improve patient care. The CLIA regulations do not expressly describe these algorithmic-only services, and clarifying that CLIA is the applicable regulatory framework for such services will ensure there is no confusion on this point, thereby encouraging laboratories to develop and leverage these innovative and groundbreaking services for the benefit of patients. As described in the section on reimbursement below, this clarification also will help to ensure that these services are appropriately coded, appropriately valued and covered by payers. Concurrent with establishing this clarity, it may be appropriate to add new

[7] See https://www.hhs.gov/hipaa/for-professionals/faq/354/does-hipaa-protect-genetic-information/index.html

specialties to CLIA—with specialty-specific quality requirements, such as expectations for personnel—to account for these innovative laboratory services.

Second, CMS should codify its enforcement discretion policy that permits pathologists and other laboratory personnel to review digital laboratory data, digital results and digital images ("digital materials") remotely—such as via a secure connection on a laptop or desktop computer in a home office—without obtaining a separate certificate. This policy of enforcement discretion, which still requires the remote work to be performed under the CLIA certificate of the primary site, was initially adopted in the early days of the COVID-19 pandemic and was instrumental to ensure the continuity of laboratory operations while prioritizing the safety of laboratory workers. Today, this enforcement discretion policy continues to be vital to laboratory operations as the industry continues to face a persistent workforce shortage.

While we applaud the agency for continuing this common-sense enforcement discretion policy[8], we strongly encourage the Administration to codify the policy in regulation so that laboratories can confidently rely on it for the future. The policy also should be expanded so as not to exclude remote review of digital cytology slides, enforcement discretion for which is scheduled to expire in March of this year.

To effectuate the above recommendations, we urge the Administration to revise the CLIA regulations as recommended in **Appendix A** to this letter.

Reimbursement: Pathways to Reimbursement and Guardrails for Use of AI in Coverage Decisions

For AI innovations in healthcare to meet their full potential, there must be a business case for bringing these services to market, including predictable and sustainable pathways to coverage and payment. Clinical laboratory services, including genetic testing, are paid under the Medicare Clinical Laboratory Fee Schedule (CLFS). Other laboratory services, such as anatomic pathology services, are paid under the Medicare Physician Fee Schedule (PFS) because they require physicians' professional time to complete the service. For both payment systems reimbursement should reflect the clinical value of the services and the resources required to furnish them. This principle must be applied in a manner that accommodates AI-enabled laboratory services. Without appropriate reimbursement, even clinically validated and widely supported AI-enabled services cannot be adopted or sustained over time.

As policymakers are in the early stages of considering how government programs like Medicare and TRICARE will pay for services that encompass AI, it is important to evaluate sections of federal law that may be outdated or too rigid to accommodate rapidly developing technology. One AI innovation in laboratory medicine takes the form of algorithmic analyses applied to data generated from existing "wet-lab" tests, often, combined with information gleaned from a medical record in an EHR.

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[8] REVISED: Clinical Laboratory Improvement Amendments of 1988 (CLIA) Post-Public Health Emergency (PHE) Guidance | CMS

These AI-enabled algorithmic services, which require substantial investment to develop, validate, and maintain, can meaningfully improve patient care by increasing diagnostic precision, supporting more informed treatment decisions, and reducing unnecessary downstream testing or ineffective therapies. The importance of this information, and the investment in developing the technology that facilitates the prediction, should be reflected in the reimbursement the laboratory receives for the service—even if the laboratory testing does not require additional specimen collection, which is currently the case under the CLFS.

Reimbursement for AI-enabled services also should not be impeded by unnecessary administrative barriers imposed by private organizations. Accurate and appropriate coding plays a foundational role in enabling coverage, payment and continued investment in clinical services, including AI-enabled laboratory analyses. When private coding governance has the effect of limiting Medicare payment pathways, creating misalignment with federal regulatory frameworks, or constraining patient access to clinically meaningful services, more proactive federal engagement with relevant coding authorities would help ensure alignment with the AI policy objectives outlined in this RFI.

One clear example of this challenge is the treatment of Proprietary Laboratory Analysis (PLA) codes. Clinical laboratories have previously received PLA codes for algorithmic-only analysis services, demonstrating that the existing coding framework has historically accommodated innovative laboratory services that rely on computational analysis rather than new wet-lab procedures. Recent changes to PLA guidance, however, have created a foreseeable coding gap for laboratory algorithmic-only analyses, as these services are expressly no longer eligible for PLA coding as of January 1, 2026. The PLA coding process was specifically designed to identify and manage innovative and proprietary laboratory tests, including novel services that do not fit neatly within existing CPT categories. Algorithmic-only laboratory analyses fall squarely within this design intent and should continue to be eligible for PLA coding as it provides a suitable and effective mechanism for identifying and reporting these services. CMS should work with the American Medical Association (AMA), which owns the Current Procedural Terminology (CPT) code set, to ensure that the PLA coding process remains available for new algorithmic-only laboratory services.

Recent proposals by the American Medical Association (AMA), which owns the Current Procedural Terminology (CPT) code set, have suggested that algorithmic-only services by laboratories should be coded as "devices", and that they may require FDA authorization to be eligible for certain types of codes. As noted above, however, in March 2025, a federal district court rules that laboratory developed testing services are not medical devices, and the FDA does not have the authority to regulate them as such.[9] Importantly, the court decision did not suggest that laboratory algorithmic services are somehow beyond the scope of this ruling, and accordingly, the PLA coding process remains appropriate for algorithmic-only laboratory services. We therefore

[9] *ACLA v. FDA*, Civil No. 4:24-CV-824-SDJ, (E.D. Tex. 2025); 90 Fed. Reg. 45134 (Sept. 19, 2025) (final rule removing language from invalidated regulation).

respectfully request that CMS work with the AMA to ensure that the PLA coding process remains available for new algorithmic-only laboratory services. Amending the CLIA regulations to clarify that a "laboratory" includes a facility performing algorithmic-only analyses of patient-specific digital laboratory information, as recommended in the "Regulations" section above, may also help to expedite resolution of this misalignment of private coding policy with federal regulatory frameworks.

Guardrails for Use of AI in Coverage Decisions: AI systems should provide transparency that enables users to understand the validity, reliability, and trustworthiness of the system. ACLA members are committed to designing trustworthy AI services, identifying any limitations of data, and clearly communicating such limitations. While AI can lead to workflow efficiencies in the health care system, it is important that it is used appropriately and judiciously. ACLA is concerned with the application of AI and other automation that negatively impacts coverage and reimbursement for laboratory services. For example, payors and third-party organizations with which health plans contract, such as laboratory benefit managers (LBMs), apply opaque front-end proprietary coding edits that often lead to automatic denial before a plan has an opportunity to review a claim for coverage based on medical necessity. Front-end edits result in a significant number of inappropriate claim denials and are a tremendous source of administrative burden on laboratories and on ordering providers. Claims must be re-submitted despite each of the laboratory testing services being medically necessary based on medical policy. AI and automated systems should not be used to deny or impede patient access to services automatically. There must be a human evaluation to ensure patients are not being denied care that is medically necessary to address their health conditions.

Research and Development: Fueled by Adequate and Predictable Reimbursement and Regulatory Clarity

Research and development for AI in clinical laboratories is driven by adequate and predictable reimbursement and regulatory clarity. When reimbursement is stable and sufficient, laboratories are better positioned to invest in developing and adopting new technologies. And when regulatory expectations are clear and appropriate, innovators can more confidently advance the field, both in terms of research and adoption.

First, research, development, and adoption of AI requires significant infrastructure, much of which represents a new expense for many laboratories. Accordingly, as laboratories seek to adopt and invest in new technologies, like AI, they must have appropriate, stable, and predictable reimbursement, reflective of the value of diagnostics to patient care. Laboratory test results are the "GPS" of health care, informing roughly 70 percent of medical decisions at a modest overall cost to the health care system. The CLFS accounts for less than 1 percent of total Medicare spending.[10,11]

[10] MedPAC Payment Basics, Clinical Laboratory Services Payment System (Nov. 2025).
[11] Center for Disease Control and Prevention: https://www.cdc.gov/lab-week/about-archive.html.

Implementation of the Protecting Access to Medicare Act (PAMA) in 2014 resulted in significant payment cuts to tests paid under the CLFS, which have not seen a payment adjustment since 2020, following three years of up to 10% payment cuts to reimbursement rates for some tests in 2018-2020). Foundational flaws prevent the CLFS under PAMA from being a market-based fee schedule with rates that are predictable, sustainable, and sufficient. The Reforming and Enhancing Sustainable Updates to Laboratory Testing Services (RESULTS) Act (S. 2761/ H.R. 5269) would mitigate further payment reductions and enact a long-term solution that would enable sustainable reimbursement rates to provide laboratories with the stability of appropriate reimbursement needed for investments into advanced diagnostic technologies, such as AI.

Second, updating and clarifying applicable regulations—such as CLIA—are essential to fostering an environment that encourages research and development of new AI technologies for clinical care. Clarifying that algorithmic-only analyses of patient-specific digital laboratory information is included under that regulatory framework will encourage further research and development by laboratories, and it will unlock barriers to patient access to these innovative technologies.

Third, the Administration should continue to support, and even take additional action, to actively encourage de-identification under HIPAA as a means to support AI innovation. With respect to HIPAA, ACLA believes that safeguarding patient data and privacy is paramount to maintain trust and uphold ethical standards in healthcare. Simultaneously, utilizing deidentified data for AI innovation enables advancements in diagnosis, treatment, and personalized medicine, ultimately improving patient outcomes. It is important to strike a thoughtful balance between protecting sensitive health information and permitting the use of deidentified data for research and development purposes to further innovation.

ACLA appreciates the opportunity to provide these comments, and we thank you for your consideration.

Sincerely,

Susan Van Meter

Susan Van Meter
President, ACLA

Enclosure: Appendix A; ACLA Artificial Intelligence Policy Paper

Appendix A: CLIA Regulation Recommendations

For the reasons described in the letter, ACLA respectfully requests that the CLIA regulations be amended as follows:

1. Amend 42 C.F.R. § 493.2 to add the following definition for "digital laboratory information" and to revise the definition for "laboratory" (revisions in **bold**):

   "*Digital laboratory information*:

   (1) Means any of the following:

   (A) A digital image derived from a glass slide; or

   (B) Data or digital images including, but not limited to, flow cytometry plots; cytogenetic karyograms; chromatographic, mass spectrometric, clinical chemistry, immunological, and hematologic data; electropherograms; gel images; and genetic expression, array and sequencing data; or

   (C) Results from software-only analysis of the information described in (A) or (B); and

   (2) Is patient-specific when it is accompanied by information that can be used to identify the individual from whose specimen the information was derived.

   …

   Laboratory means a facility for the biological, microbiological, serological, chemical, immunohematological, hematological, biophysical, cytological, pathological, **algorithmic,** or other examination of materials derived from the human body**, including patient-specific digital laboratory information,** for the purpose of providing information for the diagnosis, prevention, or treatment of any disease or impairment of, or the assessment of the health of, human beings..."

   …

2. Amend 42 C.F.R. §§ 493.43(b) and 493.55(b), which requires separate CLIA certificates for each place where laboratory testing occurs, by adding at the end the following exception:

   "(4) Laboratories, from which a board-certified pathologist or other laboratory professional accesses the designated primary site system using a secure remote access protocol to retrieve, review, and analyze patient specific digital laboratory information."

3. Amend 42 C.F.R. § 493.1274(a), the standard for cytology, to read as follows (revisions in **bold**):

   "(a) Cytology slides. All cytology slides must be prepared on the premises of a laboratory certified to conduct testing in the subspecialty of cytology."